Case study: Ethan

Condition: **Cerebral palsy – spastic quadriplegia (GMFCS IV, MACS IV)**

32-year-old man · Supported independent living home, NDIS-funded

Ethan Clarke is a 32-year-old man with **spastic quadriplegic cerebral palsy** who lives in NDIS-funded supported independent living in Ipswich with two housemates. He is classified at GMFCS level IV and MACS level IV. Ethan drives a powered wheelchair with a joystick and usually communicates using a speech-generating device mounted on his wheelchair tray, which he operates with his right hand through a keyguard. Without the device, he nods for "yes" and turns his head away for "no". He works two days a week at a community radio station, helping to programme music and record station announcements.

Ethan's last seating review, 18 months ago, documented "good postural alignment, skin intact". The house's progress notes describe him as eating well and "medically stable", and his GP saw him last month for repeat prescriptions.

You have been asked to complete a wheelchair seating review because the house manager has reported that one armrest is loose and that Ethan has been "sliding forward" in his chair. When you arrive, the house manager hands you a repair quote and asks you to sign it today. Ethan greets you with a big grin and gestures towards his Brisbane Lions scarf. Shortly afterwards, his mother rings the house phone asking to speak with you about his NDIS plan review next month.

You notice that the screen of Ethan's speech-generating device is cracked and blank. Jade, a support worker, tells you it "got dropped a couple of weeks ago" and that "the manager's sorting it". During the review, Ethan repeatedly bangs his right hand on his tray and keeps shifting his weight, leaning over to one side. Jade sighs. "He's been aggressive like this all week," she says. "Could you recommend wrist straps for the chair to stop him hitting? It'd make things so much easier for the staff." Another staff member walking past adds, "He's been like this before. It's just behaviour."

Using yes/no questions, you ask Ethan whether something is hurting. He nods. When you ask about his back, he turns his head away. When you ask about his bottom, he nods firmly and hits his tray again.

You and Jade use the ceiling hoist to move Ethan onto his bed so that you can check his seating measurements. As his trousers are adjusted, you see a purple area, about the size of a 50-cent coin, over the base of his spine, with a small open, weeping centre. Jade looks surprised. "Night staff do his personal care," she says. You also notice that the cushion in his wheelchair is not the type recorded at his last review. Jade explains that a new cushion from a different supplier was delivered three weeks ago because the old one "was worn out".

There is no clear documentation regarding who prescribed or set up the new cushion, Ethan's repositioning routine, skin checks, continence care, whether the device repair has been requested, or whether Ethan has a behaviour support plan.

Jade checks her watch and tells you that Ethan's maxi taxi will arrive in fifteen minutes to take him to his four-hour shift at the radio station. "He gets really upset if he misses work," she says. The house manager reappears and asks again for your signature on the armrest quote. Ethan looks straight at you and hits his tray.

You are the occupational therapy student completing Ethan's wheelchair seating review and your clinical supervisor is not currently in the room.

Present Ethan's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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